

Conflict of interest, delegation, and the handling of my personal information

Section 89, Public Sector Act 2022 · Code of Conduct clause 1.2

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital, Metro South Health

Employee reference MSH-INJ-5795

To: The Chief Executive, Metro South Health

Cc: Human Resources, Logan and Beaudesert Health Service — LBH.HRTeam1@health.qld.gov.au

Injury Management — lbh_injurymanagement@health.qld.gov.au

Ms Emily Petering, Industrial Officer, Together Queensland

Date: 3 August 2026

1. WHAT I AM ASKING, AND WHAT I AM NOT

I am asking that the decisions being made about my capacity, my attendance and my return to duty be made by an officer outside Corporate Services, and that the clinical material be held only by the Injury Management team.

I address this letter to you because section 89 of the Public Sector Act 2022 makes the chief executive the person to whom a conflict is disclosed, and the only person who may authorise an officer to continue to act in a matter affected by one. I do not raise it with you in order to escalate it. I raise it with you because, on the terms of the section, there is no one else who can answer the questions at Part 3.

I am not asking that anyone be removed from the management of my employment. I am not asking that anyone be stood aside, investigated, or found to have done anything wrong. I make no allegation against any individual and I seek no finding about anyone.

This is not a personal complaint. It is about how the process is being run when an employee is diagnosed with an illness and needs managerial support and reasonable adjustment.

Clause 1.2 of the Code of Conduct for the Queensland Public Service records that having a conflict of interest is not of itself wrongdoing, and that what matters is whether it is disclosed and managed. I proceed on that basis, and I ask these questions in that spirit.

2. THE POSITION AS I UNDERSTAND IT

- (a) The letters of 31 July 2026 are written in the first person of the decision-maker — "I am seeking further clarification", "I require further information", "I will make a decision regarding your ability to perform your role" — and are signed by Mr Scott Hughes, Director, Corporate Services.
- (b) Question 3 of the attached request asks my general practitioner to advise whether I am fit to return "under the existing reporting arrangements, including working with and reporting to [my] current line manager". Those reporting arrangements sit within Corporate Services.
- (c) The Manager, Switchboard Services — who on 2 July 2026 advised that my return could not be facilitated, and asked me to wait for advice as to my next returning shift — reports within that same directorate.
- (d) On 4 October 2025 I made a formal written request for review to the A/Director, Human Resources Business Partnering, concerning a process initiated by the officer who has signed the letters of 31 July 2026. My union was copied. I do not reopen that matter and I make no submission about it. I record it only because it bears on whether a conflict exists and how it must be managed.
- (e) The letter of 7 July 2026 records that my treating practitioner had advised of "unresolved workplace matters involving line management", and that the Health Service did not have sufficient information to assess whether they presented psychosocial hazards.

The officer determining my return to duty is therefore the Director of the directorate in which the advice of 2 July 2026 was given, to whom the officer who gave it reports, and whose own directorate is the subject matter of the reporting-arrangements question at question 3.

I say only that this is a set of circumstances in which a conflict may exist. Whether it does, and how it has been managed, is not something I can determine.

2A. WHAT THE "UNRESOLVED WORKPLACE MATTERS" ARE, AND WHY THEY ARE WORKPLACE MATTERS

The letter of 7 July 2026 records that the Health Service does not have sufficient information to understand the nature of the unresolved matters involving line management, or to assess whether they present psychosocial hazards. I take that at face value, and I would rather close the gap than argue about it.

The matters concern how the work is organised and decided, not the character of any officer. They are, in substance:

- (a) how patient safety concerns raised at the switchboard are escalated and answered;
- (b) how changes to the work, the roster and the on-call arrangements have been introduced, and whether affected staff were consulted before rather than told afterwards;
- (c) fatigue and recovery between shifts, and how rostering is managed against it; and
- (d) the ability of staff to have genuine input into decisions that affect the work — which the Managing the risk of psychosocial hazards at work Code of Practice 2022 treats as a matter of work design and organisational justice, and therefore as a psychosocial hazard in its own right where it is absent.

Each of those is a workplace matter. None of them can be answered by a medical practitioner writing about me. Sections 47 to 49 of the Work Health and Safety Act 2011 require the Health Service to consult workers who are directly affected when identifying hazards, when deciding how to control them, when proposing changes affecting health or safety, and when deciding the procedures for monitoring the health of workers. That consultation has not occurred at any point since 26 June 2026.

I record, without argument, that the information gap the Health Service identifies is one only the Health Service can close, and that it is being used in the meantime to maintain my exclusion.

2B. MANAGERIAL SUPPORT AND ADJUSTMENT WHILE I AM UNWELL

When an employee is diagnosed with an illness and is certified fit with restrictions, the Health Service's own policies contemplate managerial support and reasonable adjustment so the person can continue or return to work safely.

- (a) HR Policy G03 provides at clause 1.1 for temporary adjustments so that an employee may "continue work during their recovery", and at clause 1 that managers are responsible for ensuring that work environments and work "do not unreasonably prevent employment for people". Clause 1.1.1 extends workplace rehabilitation support to employees with "no claim or entitlement to compensation (health management case)".
- (b) The Workplace rehabilitation standard QH-IMP-401-5 requires that, where alternate duties are required, meaningful suitable duties be identified and actively sought and "made available wherever reasonably practicable"; that the line manager "facilitate the injured workers on-going connection with the workplace"; and that the worker be informed of the appropriate dispute resolution process. It applies to a worker requiring rehabilitation for "either a work or non-work-related injury/illness".

I obtained the checklist the Health Service required in order to facilitate my return. It certified me fit with restrictions. What followed was exclusion rather than adjustment. Interim duties were declined in any capacity on 15 July 2026, and no adjustment plan has been identified to me at any time.

I do not say that the Health Service may not seek medical information. It may. But a medical process clarifies functional limits; it does not displace the duty to consider adjustments in the meantime. I ask that interim adjustments be identified and implemented under the restrictions already certified, while the balance of the process is completed.

2C. DUE PROCESS — HOW THE DECISIONS ARE MADE

Support and adjustment are the substance. Due process is the way the decisions about them are made. In this setting, put plainly, it means:

- (a) that I am told what power is being used, and who is deciding;
- (b) that I have a fair opportunity to respond before a serious employment consequence — exclusion from the workplace, a decision about my capacity, or the stopping of my pay;
- (c) that the decision is made by an officer independent enough of the matters in issue;
- (d) that I am consulted on the workplace and psychosocial matters said to bear on my return, as sections 47 to 49 of the Work Health and Safety Act 2011 require; and
- (e) that deadlines and consequences are not one-sided.

On the last point I record only this. On 7 July 2026 and again on 31 July 2026 I was given seven calendar days, with a consequence attached. On 15 July 2026 the Health Service advised that it was unable to commit to a timeframe of its own, and on 30 July 2026 that it was unable to guarantee my return. I have asked several times for a date and have not been given one.

The Managing the risk of psychosocial hazards at work Code of Practice 2022 calls the same idea organisational justice, and treats its absence — including a lack of procedural fairness — as a psychosocial hazard in its own right. HR Policy G03 clause 1 requires that proper consideration be given to that Code in providing reasonable adjustments.

I do not say that any tribunal has found a denial of fair process, and I make no such claim in this letter. I say that the process described above has not been applied to me, and I ask that it be applied now — which is what the separation proposed at Part 4 is for.

3. THE QUESTIONS I ASK

Section 89(1) of the Public Sector Act 2022 provides that if a public sector employee has an interest that conflicts or may conflict with the discharge of the employee's duties, the employee:

"(a) must disclose the nature of the interest and conflict to the employee's chief executive as soon as practicable after the relevant facts come to the employee's knowledge; and (b) must not take action or further action concerning a matter that is, or may be, affected by the conflict unless authorised by the employee's chief executive."

I ask the Health Service to confirm:

- (a) whether any disclosure has been made under section 89(1)(a) in relation to the decisions concerning my attendance, my pay and my return to duty;
- (b) whether any authorisation has been given under section 89(1)(b) for an officer to take, or to continue to take, action concerning those matters;
- (c) who holds the delegation for those decisions, and the source and instrument of that delegation;
- (d) whether the advice of 2 July 2026 and the decision of 3 July 2026 were made with the knowledge or approval of the officer now determining my return to duty; and
- (e) whether any conflict has been identified and recorded, and if so, how it is being managed.

I ask these questions in writing because I have asked them before, in my email of 28 July 2026, and have not received an answer.

If no authorisation has been given under section 89(1)(b), I ask that you consider the direction contemplated by section 89(2), and that in the meantime the decisions described at Part 2 be allocated to an officer outside Corporate Services.

I am conscious that this is a request that you exercise a function under the Act, and not a request that anything be found about anyone. If the answer to each of (a) to (e) is that the matter has been dealt with, that answer will resolve it entirely and I will take it no further.

4. THE SEPARATION I PROPOSE

I propose the ordinary separation of functions. It requires no restructuring, removes no one from the management of my employment, and requires no finding to be made about anyone.

- (a) THE CLINICAL MATERIAL — held exclusively by the Injury Management team, Human Resources, on a medical-in-confidence basis. Corporate Services would not hold, and would not have access to, the clinical material. This is what the Employee Capabilities Checklist itself recommends at Additional Comments 4(c): "return-to-work coordination and correspondence through the Injury Management team, Human Resources."
- (b) THE DECISION — any decision concerning my capacity or my return to duty made by an officer outside Corporate Services.
- (c) IMPLEMENTATION — Corporate Services retains the operational role: rostering me, putting in place whatever adjustments are decided, and the ordinary day-to-day management of my work. My line manager would receive the adjustments to be implemented, not the clinical information behind them.

The officer steps out of the decision, not out of the workplace.

Metro South Health is a large organisation and I do not anticipate that this would present any difficulty.

5. MY PERSONAL INFORMATION

The information sought by the request of 31 July 2026 is sensitive health information about me.

- (a) I ask that it be held by the Injury Management team and used only for the purpose for which it was collected — the assessment of my capacity and any adjustments — and not for any other purpose.
- (b) I ask to be told who within the Health Service has had access to the medical material already held about me, and who it is proposed will have access to the further material when it is received.
- (c) I ask that access be limited to those who require it for the purpose, and that the clinical material not be provided to Corporate Services. What Corporate Services requires in order to roster me and implement adjustments is the adjustments themselves, not the diagnosis, the history or the clinical reasoning behind them.
- (d) I ask to be told whether it is proposed to provide the information to any person outside the Health Service, and if so, to whom and for what purpose.
- (e) I do not consent to the material being used in connection with any other proceeding.

I make these requests under the Information Privacy Act 2009 and the Queensland Privacy Principles, and consistently with the consent I have given, which is scoped in my response of the same date.

6. WHY IT MATTERS TO THE DECISION ITSELF

I raise one further matter, neutrally.

A decision on my request for a change in the way I work may only be made, or refused, on reasonable grounds. Whether grounds are reasonable is not only a question of what was decided; it is also a question of who decided it and on what material. If the decision is made by an officer within the arrangements the decision concerns, that is a matter which can be raised later, by either of us, and I would rather it were dealt with now.

I would also note that the Managing the risk of psychosocial hazards at work Code of Practice 2022 identifies poor organisational justice — including a lack of procedural fairness — as a psychosocial hazard in its own right, and that HR Policy G03 requires proper consideration to be given to that Code.

I therefore ask that this be resolved before the decisions are made, rather than after.

6.1 ONE MATTER I RECORD FOR THE FILE

I record the following once, neutrally, and I do not develop it here.

In the course of my employment I have raised concerns about patient safety, about changes to the work and the roster, about fatigue, and about the ability of staff to have input into decisions affecting the work. I am entitled to raise those matters, and to be consulted on work health and safety matters that affect me.

Since 3 July 2026 I have been held out of the workplace, my accrued leave has been debited, and from 13 July 2026 I have been without wages, while those matters remain unassessed by the Health Service and while the decisions about my return sit within the directorate they concern. That is a serious detriment to me.

I am not making a claim in this letter and I ask for no finding. I record it because it is a matter I have raised with my union, and because I would rather it be dealt with by the Health Service now than become an issue later. I do not abandon my employment or any entitlement.

7. WHAT THIS LETTER DOES NOT DO

- (a) It makes no allegation against any individual and asks for no finding about anyone.
- (b) It does not ask that anyone be removed from the workplace, stood aside or investigated.
- (c) It is separate from my response of the same date to the Request for Medical Information, from my request under clause 10.3.2, and from my notice of dispute under clause 1.11. A decision on any of those is not an answer to this.
- (d) It is not a complaint under any Act, and it is not a grievance.

I confirm again that I am engaging with this process and want to return to work.

Yours sincerely,

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital

